



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : ام هاشم محمود احمد موسي

العنوان : المنيا

الحي / المدينة : دير مواس

المحافظة : المنيا

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 27309112400268 رقم التليفون : 01222222654

العمر (تاريخ الميلاد) : 11-09-1973 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ام هاشم محمود احمد موسى National ID: 27309112400268	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

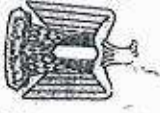
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٢/٨
١٧٣٠٩١١٢٤٠٠٢٦٨
رعية منزل
التي
عائل مختل عائلتين مختل
الطاقة سارية حتى ٢٠٢٢/٨/٢٤



die gie

uscip

cshicn

ahka
7/9/19

ERM036745564



VI
ce

التي
١٧٣٠٩١١٢٤٠٠٢٦٨
die gie



JR7225802

جمهورية الأردن
وزارة الداخلية
التي
عائل مختل عائلتين مختل
الطاقة سارية حتى ٢٠٢٢/٨/٢٤

MDT FORM

/ / 20

G

Name	ام وائل محمد دارة	MRN	1815/2025
Age	52	ID	27307091701302
Menopausal status	Post menopause	Phone	01558714261 01559215049
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	Post op

Clinical Data

BX: Lt. + DC CII

ER: 018 PR: 318 HER 2: 3+ Ki67:

Sonomammography:

S/P Lt mRM (post op)

Mets Workup: minimal (x-) residual low grade

Ductal Carcinoma in situ

- No residual viable remnants of the previously diagnosed invasive tumor
- Decol margin -> free

Recommendations: nipple parathy -> free

Adjuvant (0115).

Pathological complete response

MDT OUTCOME & PLAN

① Complete one year phasgo

② PORT (initially T2N1M0)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد عبد الحليم 2025/11/10	د. محمد عبد الحليم 2025/11/10	د. محمد عبد الحليم 2025/11/10			

Patient Name : الأستاذة / ام هاشم محمود احمد موسى	Request No. : 13004079
Patient No. : 27309112400268	Request Date : 21/12/2025
Path No. : 3921/25	Verification Date : 31/12/2025
Sex : Female	Financial Category : جوستافا روسي - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature Of Specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 30x22x5 cm, with covering skin ellipse measured 26x22 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 10x8x3 cm, located 2 from the least deep margin.
- * Axillary fat measured 10x7x2 cm, entangling 15 lymph nodes, the largest measured 2x1x0.5 cm.
- * 17 Blocks:
- 1:10- T.B.S., 11- N&A., 12- Deep margin., 13:17- ALNs.
- Gross was done by: Dr. Heba El-Shafeey.

Microscopic Examination:

- * Histological type: Minimal (1%) residual low grade ductal carcinoma in situ (Cribriform pattern).
- * No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%)
- * Angioinvasion: Absent.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Fifteen lymph nodes free of viable metastatic deposits (0/15) (Some lymph nodes showing effect of therapy).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:
- Minimal (1%) residual low grade ductal carcinoma in situ (Cribriform pattern).

Verified by :

Dr.Duaa Mohamed

Printing Date : 04/01/2026 09:08:39

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



شركة رقم (4) - الدور الاول - مبنى رقم (937) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Msr El Qadema - Cairo- Egypt

Patient Name : الأستاذة / ام هاشم محمود احمد موسى
Patient No. : 27309112400268
Path No. : 3921/25
Sex : Female
Age : 52 Y
Referring Doc :

Request No. : 13004079
Request Date : 21/12/2025
Verification Date : 31/12/2025
Financial Category : جوستاف روسي - بطاقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- ⊙ No residual viable remnants of the previously diagnosed invasive tumor.
- Deep Margin: Free.
 - Nipple & areola: Free.
 - Axillary lymph nodes: Fifteen lymph nodes free of viable metastatic deposits (0/15) (Some lymph nodes showing effect of therapy).
 - Histologic response to therapy: Pathological complete response to therapy.
 - Stage: ypTis, ypN0.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafey.

Verified by :
Dr. Duaa Mohamed

Printing Date : 04/01/2026 09:08:39

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



جريدة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / ام هاشم محمود احمد موسى
Patient No. : 27309112400268
Path No. : SR-878/25
Sex : Female
Age : 51 Y
Referring Doc :

Request No. : 13003480
Request Date : 29/06/2025
Verification Date : 30/06/2025
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Left breast mass

Nature of specimen

Block & slide revision

Gross examination

One block labelled by (201964/25) & three immuno-stained slides were received

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+3+2) Grade 3
- * DCIS: Absent
- * Angioinvasion: Absent
- * Perineural invasion: Absent
- * IPT review slides :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: Weak nuclear reaction in 10 % tumor cells (Allred score 3/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)

Diagnosis

Left breast mass, block & slide revision:

Invasive duct carcinoma, Grade 3

IPT revision:

- * ER: Negative (score 0/8).

Verified by :

Dr.Heba Hasan

Printing Date : 05/07/2025 10:17:35

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAP

Patient Name : الأستاذ / ام هاشم محمود احمد موسى

Patient No. : 27309112400268

Path No. : SR-878/25

Sex : Female

Age : 51 Y

Referring Doc :

Request No. : 13003480

Request Date : 29/06/2025

Verification Date : 30/06/2025

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

PR: Positive(score 3/8).

HER2: Positive(score 3+).

Signature

Dr. Walaa Ibrahim Ghoneim

Dr.Heba Hassan Adly

Verified by :

Dr.Heba Hasan

Printing Date : 05/07/2025 10:17:35

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 287- Nile Corniche - Misr El Qadema - Cairo, Egypt

MRN 20225020080
Patient Name: OMHASHEM MAHMOUD AHMED MOSA

D.O.B: 11/09/1973
IC No: 27309112400268
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 992506160086
Accession No Z-IMM25-202177
Sample Type Paraffin Block

Collection Date 16/06/2025 01:50 PM
Received Date 16/06/2025 01:51 PM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

201964-25

Clinical Data

Lt. breast cancer.

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :

ER: Negative, [0/8].

PR: Negative, weak nuclear reaction in <1% of tumor cells [2/8].

Signature

Heba Abdallah

Reported and electronically endorsed by

Hazem Abdulazim Sobhy Abo Ismael

MRN: 20225020080
Patient Name: OMHASHAM MAHMOUD AHMED MOSA

D.O.B: 11/09/1973
IC No.: 27309112400268
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 992506160085
Accession No Z-IMM25-202176
Sample Type Paraffin Block

Collection Date 16/06/2025 01:50 PM
Received Date 16/06/2025 01:51 PM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

201964-25

Clinical Data

Lt. breast cancer.

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

left

Laterality - 1

Left

Her2 Result - 1

Positive [score 3+] —

Signature

Heba Abdallah

Reported and electronically endorsed by
heba ahmed Abdel Baqi Abdallah

MRN: 20225020080
Patient Name: OMHASHEM MAHMOUD AHMED MOSA

D.O.B: 11/09/1973
IC No.: 27309112400268
Sex: Female
Location: External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 992506140099
Accession No Z-SUR25-201964
Sample Type Tissue

Collection Date 14/06/2025 03:27 PM
Received Date 14/06/2025 03:28 PM
Doctor Nada Elsayed Sabry

Clinical Information

Left breast LOQ malignant looking lesion BIRADS 5.

Gross Picture

Five cores 1-1.2 cm. .

Microscopic Picture

A case of invasive duct carcinoma, grade 3. No associated DCIS. No lymphovascular emboli. Stroma is desmoplastic with marked inflammatory cell reaction. TILS 30%.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, grade 3

Laterality

Left

Specimen type:

Tru-cut biopsy.

Location

Lower outer quadrant

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Diagnosis

Left breast, TRU-cut biopsy:

Invasive duct carcinoma, grade 3

Comments

ER, PR & HER-2 are ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

MAHITAB IBRAHIM MOHAMED PHD

Co-signed by:

Name: Mrs. OM HASHIM MAHMOUD
Date: 3-12-2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
Clinical summary: KNOWN LEFT BREAST CANCER , underwent NAT , follow ups study.

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Evidence of benign macrocalcification.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- scattered partially obscured densities.
- LIQ grouped suspicious microcalcifications
- Ultrasound demonstrated 5 o'clock zone B , ill-defined irregular hypoechoic area of parenchymal distortion measures 9 x 5 mm .
- Heterogeneous IQ with prominent duct and 9 o'clock zone b hypoechoic oval lesion measures 5 x 2 mm
- retro areolar prominent ducts
- multiple level pathological axillary LNs (more than 3)are noted largest measures 2.7 x 1.2 cm , cortical thickening measures 3.7 mm
- no infra or supra clavicular lymph nodes.

Conclusion:-

Right breast (BIRADS 2): no suspicious findings.

Left breast (BIRADS 6): regressive course of previously noted 5 o'clock mass lesion, with IQ suspicious microcalcifications and heterogeneous parenchyma, ipsilateral pathological axillary lymph nodes .

Best Regards,

DR. RANIA AHMED, MD

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with

Category & recommendations	recommendations: Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
	A: mild suspicion.
	B: moderate suspicion.
	C: high suspicion.
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Patient Name: Om Hashem Mahmoud Ahmed Mousa	Date: 23/07/2025
Exam: MRI LUMBER SPINE	Age: 52 Year(s)
Medical ID:	Pat. ID: 110671

MRI OF LUMBAR SPINE

Findings:

- Spondylodegenerative changes in the form of diminished bright T2 signal intensity of intervertebral discs, schmorl's nodes and modic changes.
- L1/L2 and L2/3 biforaminal disc protrusion are seen encroaching upon lateral recess of both exit neural foramina.
- L3/L4 mild disc bulge about mm is seen effacing the ventral epidural space.
- L4/5 disc bulge is seen barely touching the ventral thecal aspect as well as encroaching upon both exit neural foramina ..
- L5/S1 central broad base disc protrusion is seen indenting the ventral thecal aspect as well as encroaching upon lateral recess of both exit neural foramina .
- Normal size and signal intensity of the dorsal spinal cord with no evidence of cord atrophy, malacia or syrinx formation.
- Normal appearance of conus medullaris
- No evidence of intra/para spinal masses.
- Normal marrow signal of the examined vertebral bodies and their appendages with no evidence of vertebral wedging or collapse.

Opinion:

- L1/L2 and L2/3 biforaminal disc protrusion
- L3/L4 mild disc bulge
- L4/5 disc bulg
- L5/S1 central broad base disc protrusion

BEST REGARDS ,

DR/ Reham khedr, MD

CONSULTANT RADIOLOGIST

A. h.

PATIENT NAME: OMHASHAM MAHMOUD AHMED MOSA

GENDER: Female

DATE : 25-Jun-2025 03:19 PM

PATIENT ID: 20225020080

AGE: 51 y

ORDER: Z210457,Z210458

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH SPECIAL TRIPHASIC STUDY OF THE LIVER
TECHNIQUE:

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 90 ml.

FINDINGS:

1-CHEST

- Left breast lower outer quadrant irregular heterogeneously enhancing mass lesion is seen measuring about 2.8 x 3.9 cm along its maximum axial dimensions. LOQ small breast parenchymal nodules are also seen measuring about 11 x 8 and 8 x 8 mm along their maximum axial dimensions.
- Pathologically enlarged left axillary level I, II and III lymph nodes, the largest measures about 4.3 x 1.9 cm along its maximum axial dimensions and seen abutting the related aspect of the pectoralis minor muscle.
- Few small left supraclavicular lymph nodes, the largest measures about 8 mm along its maximum axial dimensions.
- Few small internal mammary lymph nodes, the largest measures about 4 mm along its maximum axial dimensions.
- Right upper lobar tiny millimetric pulmonary nodule is seen measuring about 3 mm along its maximum axial dimensions.
- Otherwise, no sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- Middle lobar and lingular ateleatic bands.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiomegaly.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous reduced CT texture denoting fatty infiltration. No pathologically enhancing hepatic focal lesions noted throughout different study phases.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches.
- Calcular gall bladder with no peri cholecystic edema or fluid collections.

PATIENT: OMHASHAM MAHMOUD AHMED MOSA, 20225020080, Female, 51 y

Report version 2, 25-Jun-2025 05:23 PM

Page 1 of 2



www.Baheya.org



16602



Pat Name: *أحمد محمد أحمد*

Pat ID: 501949

Date: 16/07/2025

Pat. Age 51 Year(s)

Ref.Prof.Doc:

Tc^{99m} MDP bone scan

Clinical data:

- A patient with history of left breast cancer, referred for whole body skeletal assessment.

Technique:

- Delayed osseous phase whole body scan was obtained after 2 hours post injection of radiotracer.

Findings:

- Focal active tracer uptake is noted along few vertebrae (DV8, LV2 and LV3) as well as the right sacro-iliac region.
- Degenerative changes of the rest of the spine are noted.
- Arthritic changes of the big joints are noted.

Opinion:

- Few worrying osseous lesions carrying the possibility of being metastatic for CT correlation to exclude underlying serious pathology.

As regards,

Dr. Osama Elmogy, M.Sc
Nuclear Medicine Specialist

Dr. Nahla Dessoki, MD
Nuclear Medicine Consultant



Patient's Name:

Umm Hashim Mahmoud Ahmed

Date:

2026-08-31

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Rousey International Egypt

Trans-vaginal ultrasound scanning revealed: -

o Uterus: -

- RVF, average size uterus measuring about: 7x4x3cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows homogenous texture with tiny echogenic subendometrial nodule suggesting adenomyosis
- **The endometrial line is 3mm.**
- The cervix is of average size with no focal lesion

o Ovaries: -

- Both ovaries are average size with no focal lesion
- o No gross adnexal soft tissue masses or cysts.
- o No free fluid in the Douglas pouch.

Best regards

Dr. NERMEEN RAMADAN FEBR, MD

ECHO REPORT

NAME: nel goss milo p1 DATE: 14/1/2027
ID NUMBER: / SEX: ♀ AGE: 51

EF	65%.	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.9	3-3.7cm
LVEDD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions and
LV Wall Thickness increased
LV Diastolic Function DD grade I
SWMA and
LA and Ao Root Dimensions and
Mitral Valve and
Aortic Valve and
Tricuspid Valve and
Pulmonary Valve and
Right Side and Systolic Function and
Intracardiac Masses and Effusion and

DR

Shah

Patient's Name:

Umm Hashim Mahmoud Ahmed

Date:

2026-08-31

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Rousey International Egypt

Trans-vaginal ultrasound scanning revealed: -

o Uterus: -

- RVF, average size uterus measuring about: 7x4x3cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows homogenous texture with tiny echogenic subendometrial nodule suggesting adenomyosis
- **The endometrial line is 3mm.**
- The cervix is of average size with no focal lesion

o Ovaries: -

- Both ovaries are average size with no focal lesion
- o No gross adnexal soft tissue masses or cysts.
- o No free fluid in the Douglas pouch.

Best regards

Dr. NERMEEN RAMADAN FEBR, MD

ECHO REPORT

NAME: nel soussou mli DATE: 24/1/2027
ID NUMBER: / SEX: ♀ AGE: 51

EF	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.5	AO.R	2.9	3-3.7cm
LVEDD	2.4cm	LA	3.5	2.4cm
FS	25-45%	RV		0.7-2.3cm(supine)
SWT	1.2	AO.CUSP		1.5-2.6cm

LV Dimensions and
LV Wall Thickness increased
LV Diastolic Function DD grade I
SWMA and
LA and Ao Root Dimensions and
Mitral Valve and
Aortic Valve and
Tricuspid Valve and
Pulmonary Valve and
Right Side and Systolic Function and
Intracardiac Masses and Effusion and

DR

Shah